About this guide

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Clinical manuscript review

Dr. Salwa Ismael Hegazy, PhD, Clinical Neuropsychology, The American University in Cairo; Ahmed Amin, Clinical Psychologist, Okasha Institute of Psychiatry, Ain Shams University

Reviewer contributions and credentials are supplied by the author. Dr. Salwa Ismael Hegazy reviewed the revised manuscript and amendments; Ahmed Amin's contribution concerns the original manuscript. No institutional endorsement or empirical validation is claimed.

Purpose and scope

Every facilitator knows the moment: the skill is on the board, and the room has gone somewhere else. This guide is for that moment. It is written for qualified facilitators and supervised trainees who run adult skills groups in IOP and PHP programs. Each fictional case gives you a first response, choices for what happens next, a way back to the planned skill, and sample documentation.

The cases are set in rolling-admission groups with planned 50-minute sessions. Most take place in the room; four take place by video. Group size, staffing, and level of care change from case to case, as they do in real programs, so adjust each one to your own setting.

The worked skills come from dialectical behavior therapy (DBT). The practice examples are short and are meant to support a lesson you are already teaching. They do not replace a DBT curriculum, your training and supervision, or your program's procedures. The documentation examples are starting points, not complete notes, and they are not written to support medical necessity, payment, or legal compliance.

All names, cases, and dialogue are invented. The references support the principles behind each response; the scripts themselves have not been tested as a clinical protocol. The cited authors and organizations have not sponsored or endorsed this book. Each scenario stands on its own.

Acute safety and virtual groups

Scenarios 33 to 35 cover a stated plan to end one's life, apparent intoxication, and a threat toward another member. Scenarios 36 to 39 cover groups held by video. They show a facilitator's first response: what to say, how to stay with the group, and how to hand off. They are not a suicide risk assessment, crisis protocol, or security procedure, and they do not replace your program's emergency procedures.

Permission to print the cards

Program-use permission. The purchasing professional or program may make internal copies of the designated practice pages, session plans, cards and blank implementation forms for its own staff and clients directly served. Staff may use these copies within that program; a separate purchase for each staff member is not required. This permission does not allow resale, public uploading, external distribution of blank files, or distribution of the complete digital books. Other uses require written permission from Thoughtkind Press. Keep completed clinical information in approved systems. The cards and staff implementation forms are professional references, not client handouts. Permission covers original material in this publication only, not cited publishers' materials.

Safety first

Use this page before any card when safety may be in question. It does not replace your program's crisis, emergency, or medical procedures; it helps you recognize when to use them. Scenarios 33 to 36 walk through four of these situations in detail.

Stop routine teaching when you notice

• Statements of possible danger: for example, "I don't know whether I can get through tonight," talk of wanting to die or of not being safe, or a threat toward someone else.

• A change in awareness: new confusion, difficulty staying awake or being roused, slurred speech, unsteadiness, or signs of possible intoxication or overdose.

• A possible physical emergency: chest pain, trouble breathing, unusually slow breathing, fainting, or a seizure.

• A report of current danger: someone describes being unsafe now or having nowhere safe to go tonight. Establish the appropriate support response promptly; a housing need alone is not automatically a medical emergency.

Then, in every scenario

• Maintain group coverage while activating help. Never send peers to escort, find, monitor, or interpret for someone. If the room is unsafe, follow the emergency or evacuation route rather than keeping everyone seated.

• Request help now through your program's route. If nobody answers, use the backup route. Confirm the receiving person, the responsibility accepted, next action and timing. A sent request alone does not transfer responsibility.

• Protect privacy. Ask only immediate safety questions needed within your role; arrange fuller assessment privately with group coverage. Do not request a public history or detailed account.

• Do not finish the lesson first, and do not let the end of the session close the response. Ask the person to stay with you until staff connect, unless your program's procedure directs otherwise.

• For a possible medical emergency, activate the emergency response immediately within your role. Do not wait for routine staff contact or permission to finish the lesson.

• For video groups, use the confirmed location and backup number; request assistance while maintaining supervision. A camera left on is not a substitute for attention or coverage.

• If anyone is in immediate physical danger, use your program's emergency procedure at once. Your role is to get help, not to restrain anyone.

In the United States, the 988 Suicide & Crisis Lifeline (call or text 988) is available 24/7. It supplements, but does not replace, your program's response or emergency medical services when immediate medical response is needed.

When you request help, say

Who you are and where you are; what you observed and what was said; where the person is now; what you need; and that you are staying with the group.

My program's routes

Help during group: _____

Backup if no answer: _____

Emergency procedure: _____

Medication questions: _____

Language services: _____

Standalone quick reference

When one member needs more time

Scenario 1 · Adult IOP · Planned skill: DBT STOP

Notice

Ten minutes into check-in, Denise has spoken rapidly for four minutes about a housing and family conflict. Others are waiting. No overt threat has been stated; safety still requires attention.

First response

“Denise, that sounds like a hard night. I’m pausing the details so everyone has time. Would you like to ask for a staff check-in without sharing details, or listen for now?”

Choose the next step

• Engages: Offer private practice, a fictional example, or observation.

• Objects or mentions housing: Acknowledge the impact. Clarify urgency through the program’s support route; practical help and clinical assessment are different responses. Do not postpone an immediate need for the lesson.

• Peers offer advice: “Let’s hold peer advice to protect Denise’s privacy and everyone’s check-in time.”

• Goes quiet: Offer listening and a feasible check-in; quiet isn’t the same as better.

• Possible harm or medical concern: Pause routine redirection and use local assessment or emergency procedures.

Return to STOP

Confirm a feasible support step suited to the urgency before routine practice; the housing issue need not be resolved in the room. Use a separate heated-message example: Stop, Take a step back, Observe, Proceed mindfully. Pause before sending, notice the urge, and choose an action that fits the goal. Stepping back need not mean leaving. Practice is not a condition for receiving help.

Support and record

Coordinate private support and coverage with a co-facilitator. Alone, stay with the group and request staff help; never leave it unattended. Promise only available follow-up. Record actual support and participation, distinguishing observation from practice.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

Nobody answers the opening question

Scenario 2 · Adult IOP · Planned skill: DBT Describe

Notice

Nobody answers a broad question about the previous session. Two members are new. Thinking, an unclear prompt, access needs, or a participation choice are possibilities; the cause is unknown.

First response

“I asked too much at once. Let me show one example. No personal story needed, and I’ll not call around the room.”

Choose the next step

• Someone responds: Acknowledge the specific contribution; do not ask them to explain everyone’s silence.

• Still quiet: Contrast A, “The cup is beside the notebook,” with B, “Someone left it there because they don’t care.” Invite an A or B response, an indicated choice, or private consideration. Responding and writing remain optional.

• Access barrier: Adapt the example or mode and check whether it helps. Do not assume everyone can see the cup.

• Interpersonal concern: Address reported harm before using practice to move on.

• Changed awareness or responsiveness: Use local assessment and response procedures. Ordinary quiet alone is different.

Return to Describe

Model putting words to a noticed feature without adding motives. Invite another description, private practice, or observation. A thought can be labeled as a thought without disproving it. Do not keep repeating the prompt to pressure an answer.

Support and record

Coordinate any individual support and group coverage with a co-facilitator. Alone, stay with the group and request staff help as needed; never leave it unattended. Arrange feasible follow-up. Record observed responses; private practice and its effects remain unconfirmed without further information.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A disagreement becomes personal

Scenario 3 · Adult IOP · Planned skill: DBT GIVE

Notice

Elena labels Marcus during a disagreement. Marcus objects; speech overlaps. Interrupt the attack without assigning equal blame to the insult and the response.

First response

“Elena, I’m stopping the personal comment. You can disagree with the suggestion without labeling Marcus. Marcus, you don’t need to defend yourself or explain more.”

Choose the next step

• Label withdrawn: Marcus may speak about the topic, listen, or request support. Do not declare repair complete.

• Defends it: Offer listening or disagreement about the strategy without a personal label.

• Repeats the attack: Pause discussion and request staff support. Avoid debating the label.

• Peers take sides: “Hold on. I’m addressing this. We won’t ask the group to judge either person.”

• Marcus goes quiet: Offer feasible support without requiring reassurance or mediation.

• Threat or significant clinical change: Use local assessment or emergency procedures.

Return to GIVE

Resume only when attacks have stopped and support needs have been addressed. GIVE does not require tolerating mistreatment or staying in an unsafe relationship. Introduce Gentle, Interested, Validate, and Easy manner using colleagues who disagree about an agenda. Model acknowledgment and curiosity. Do not replay the members’ conflict.

Support and record

Coordinate individual support and coverage with a co-facilitator. Alone, stay with the group and request staff help; never leave it unattended. If hostility continues, stop debating, update responding staff through the available route, and follow local escalation procedures. Do not delay assessment. Record actual conduct and support in each member’s record without identifying peers.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member declines the activity

Scenario 4 · Adult IOP · Planned skill: DBT Check the Facts

Notice

Tom declines a personal worksheet and questions disclosure and invalidation. A refusal doesn't mean the member won't learn, and it doesn't call for removal.

First response

"You don't have to put your story on paper. This exercise shouldn't start by deciding you are wrong. Let me explain what happens to the pages; then we can use a made-up example, or you can listen."

State the program's actual collection and retention arrangements.

Choose the next step

• Accepts another route: Use a fictional example without required writing. Invite one contribution or listening.

• Still declines: Clarify once and respect the answer. Review unresolved participation questions separately under program procedures.

• Others also object: Explain the same available alternatives without asking peers to persuade Tom.

• Access barrier: Offer a feasible adaptation and check that it helps; do not promise unavailable materials.

• Current harm or clinical change: Take reported harm seriously rather than arguing it away; pause the exercise for support, local assessment, or emergency procedures as indicated.

Return to Check the Facts

Use an instructor moving an unfinished bowl. The fictional participant feels ashamed and anxious and anticipates rejection. Separate the action from the interpretation. Examine missing facts, other explanations, and evidence of exclusion. Consider likelihood, coping, and whether the emotions and their intensity fit what is known. Avoid invented reassurance.

Support and record

Coordinate individual support and coverage with a co-facilitator. Alone, stay with the group and request staff help as needed; never leave it unattended. Record actual choices, adaptations, contributions, and follow-up. Listening does not demonstrate mastery.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A new member joins midway through the cycle

Scenario 5 · Adult IOP · Planned skill: DBT Problem Solving

Notice

Priya arrives at the session's beginning but lacks context from earlier lessons. Lee wants application time. A lesson introduction does not replace essential admission or consent preparation.

First response

"Priya, I skipped the starting point. Let me show it. Lee, we'll keep time to put it to use too."

Choose the next step

• Finds an entry point: Acknowledge the specific contribution; one contribution doesn't mean the member is fully oriented.

• Still unclear: Simplify to one task, model accessibly, and arrange feasible teaching support. Observation remains an option.

• Returning member objects: Offer starting and application tasks to everyone. Do not rank members or ask peers to teach Priya.

• Essential preparation gap: Use the program's preparation process before sensitive participation. Obtain missing admission information through staff.

• Significant clinical concern: Use local assessment or emergency procedures with appropriate support and coverage. Basic questions alone are different.

Return to Problem Solving

Use an appointment reminder with no address. Identify the situation, check the facts, define the goal, generate options, compare and select one, implement it, and review the result. Consider access to a letter, phone, or other channel. If a phone line disconnects, find another feasible route rather than blaming effort.

Support and record

Coordinate any individual explanation and group coverage with a co-facilitator. Alone, stay with the group and arrange staff support through the established route; never leave it unattended. Confirm follow-up responsibility and timing. Record demonstrated understanding and actual arrangements without inferring readiness.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

The facilitator makes a harmful assumption

Scenario 6 · Adult IOP · Planned skill: DBT Validation

Notice

You linked Andre's unfinished practice page with not caring about recovery. Correct the assumption even without an objection. Quiet is not the same as repaired.

First response

"Andre, I assumed an unfinished page meant you didn't care. That was unfair, and I'm sorry. You don't need to explain your circumstances."

Choose the next step

• Names the impact: Reflect accurately, acknowledge your conduct, and change the invitation. Do not request forgiveness.

• Remains distrustful: Describe what will change and offer the program's actual feedback or alternative staff route. Confirm availability before promising contact.

• A peer defends you: "Intention doesn't undo what I said. Andre doesn't need to agree that it was harmless."

• Goes quiet: Offer a feasible check-in once; do not require reassurance.

• Possible harm or medical concern: Use local assessment or emergency procedures with coverage. Criticism alone is different.

Return to Validation

Ask, "Was the task useful, or did something get in the way?" Personal circumstances need not be disclosed. If appropriate, use a separate friend disappointed by a canceled event. Listen and reflect what was said; check accuracy without inventing feelings or history. Validation need not endorse every conclusion. Do not use Andre's experience as the demonstration or treat resuming as proof of repair.

Support and record

Coordinate support and coverage with a co-facilitator. Alone, stay with the group and request staff assistance; never leave it unattended. Promise only available follow-up. Seek supervision. Record your own conduct and the member's actual response, including unresolved concerns.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member repeatedly interrupts

Scenario 7 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Gloria starts speaking before several members finish. Restore access to turns without guessing intent or asking peers to manage Gloria.

First response

“Gloria, I’ll come to you next. I want to hear the end of this turn first.”

Ask the interrupted speaker whether they want to finish or pass. Keep the place in the sequence you promised Gloria.

Choose the next step

• Uses the cue: Invite the contribution when promised. A note is optional, not required.

• Overlaps again: Restate the current turn once, clarify a usable cue, and change the format if needed. Do not debate whose idea came first.

• A peer criticizes: Manage turns yourself; stop personal labels and restore the affected speaker’s choice.

• Discussion remains unworkable: Pause the open exchange. Use available support and local participation procedures, not automatic removal.

• Possible harm or clinical change: Pause teaching for local assessment or emergency procedures. Do not finish the exercise first.

Return to One Mindfully

Confirm an understandable order and an opportunity to finish or pass. Use a separate folder-assembly example: notice switching to an unrelated shopping list and return attention to the current step. Offer an accessible task or observation; silence is not mastery.

Support and record

With a co-facilitator, coordinate private support and coverage. Alone, remain with the group and request staff help. Do not leave the group unattended. Record actual interruptions, cues, responses, and confirmed or pending follow-up without unnecessary peer identifiers.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

Advice takes over another member's turn

Scenario 8 · Adult IOP · Planned skill: DBT Validation

Notice

Nia gives directives before Miles asks for ideas. Others add solutions; Miles stops speaking. Do not infer consent or require an explanation of private circumstances.

First response

"Let's pause the advice. Miles, do you want us to listen, or would you rather stop here?"

Choose the next step

• Wants listening: Invite an optional reflection of what was actually said. Check accuracy without adding a solution.

• Advice continues: State the boundary once and stop the advice exchange. Do not debate whether quitting work is wise.

• Wants ideas: Clarify scope, invite brief relevant experiences, and permit stopping. Do not endorse clinical or other instructions outside your role.

• Goes quiet or requests support: Offer listening or a feasible staff check-in without requiring personal details.

• Possible harm or clinical change: Pause teaching for local assessment or emergency procedures. Do not finish the activity first.

Return to Validation

Stop unsolicited directives and address requested support before resuming. Use a separate fictional neighbor disappointed about a full community garden. Attend, reflect the stated experience, and check accuracy. Understanding does not require endorsing every conclusion. Offer practice or observation.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request staff help; never leave it unattended. Promise only available follow-up. Record the individual's actual contribution and response without identifying peers; distinguish requested from confirmed follow-up.

Skill source: Linehan (2015), DBT Skills Training Manual, second edition. Guilford teaching notes

Standalone quick reference

A member cannot use the worksheet

Scenario 9 · Adult PHP · Planned skill: DBT Describe

Notice

Luis reports that the page is unusable. Do not label this refusal or ask for a diagnosis, reading history, or public demonstration of difficulty.

First response

“This page isn’t working. I’ll explain one step without writing. You can answer aloud, try privately, or watch.”

Stop peers from examining the page or taking over uninvited.

Choose the next step

• Oral route helps: Model one step and invite an optional contribution. Give feedback on what is demonstrated.

• Still unclear: Change the explanation or format; do not repeat it louder. Check your explanation without testing Luis publicly.

• Access remains unresolved: Arrange appropriate staff support and an actually available option. Do not claim that watching an inaccessible model meets the need.

• Sudden visual or cognitive change, possible harm, or medical concern: Pause teaching for local

assessment or emergency procedures. Do not finish the example first.

Return to Describe

Confirm a usable entry or an appropriate support step. Use a separate folded-towel example: describe an uneven edge rather than infer carelessness. Adapt the object or sensory route without requiring touch, reading, or writing. An oral detail can show one step; private practice remains unobserved.

Support and record

With a co-facilitator, coordinate private clarification and coverage. Alone, remain with the group and request help; never leave it unattended. PHP does not guarantee immediate backup. Record reported needs, actual formats, responses, and pending arrangements without diagnosing their cause.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Access source: AHRQ, Health Literacy Universal Precautions Toolkit, Tool 5. AHRQ Tool 5

Standalone quick reference

A tired member struggles with the lesson

Scenario 10 · Adult PHP · Planned skill: DBT Observe

Notice

Karen reports exhaustion and difficulty with several steps. Do not assume a cause. Answering a question does not rule out a clinical concern.

First response

“Karen, I hear that you’re exhausted. Do you need staff help now, or would a simpler way to take part be useful?”

Do not require public health details. Emergency action takes priority over this question.

Choose the next step

• Smaller task fits: Offer one accessible step without writing, or observation. Continue attending to changes.

• Feels unwell or requests help: Use the clinical assessment route promptly; another practice attempt is not required.

• Medication question: Connect with the appropriate clinician; do not advise dose changes.

• Peers judge effort: Stop the comparison without disclosing private information.

• New confusion, difficult to rouse, slurred speech, unsteadiness, or unusually slow breathing: Stop

teaching and activate local emergency procedures immediately. Do not finish the activity or wait for a break.

Return to Observe

Address indicated assessment and confirm support before routine teaching resumes. Choosing to listen does not justify delay. Use a separate cabinet-light example with an accessible sensory alternative; no closed eyes, breathing change, or movement is required. Taking part in practice doesn’t make someone medically fit to continue.

Support and record

With a co-facilitator, coordinate support, supervision, and coverage. Alone, remain with the group and request help; never leave it unattended. Use local escalation if help is delayed. Confirm handoff responsibility. Record actual observations, reported practice, requests, and pending responses.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Safety source: MedlinePlus, Confusion. MedlinePlus Confusion

Standalone quick reference

Silence after a painful disclosure

Scenario 11 · Adult IOP · Planned skill: DBT Validation

Notice

Rachel shares a recent loss, then interprets silence as rejection. Do not assume what the silence means or demand personal reactions from peers.

First response

“Rachel, I’m sorry to hear about your brother. You don’t need to make this easier for us. Would you like us to listen, or stop here?”

Choose the next step

• Wants acknowledgment: Reflect the stated meaning and check accuracy. Honor a request to stop.

• Feels rejected: Acknowledge the impact without claiming to know everyone’s feelings. Invite an optional response only if wanted.

• Peers explain or compare losses: Stop explanations and protect disclosure choices. Do not turn the moment into compulsory sharing.

• Requests support: Confirm a feasible staff route and its timing; do not promise unavailable contact.

• Possible harm or clinical change: Pause teaching for local assessment or emergency procedures. Do not finish a reflection exercise first.

Return to Validation

Acknowledge the disclosure and address immediate support needs before resuming. Use a separate canceled-choir example. Reflect what was said, check accuracy, and offer practice or observation. Rachel’s grief is not the demonstration.

Support and record

With a co-facilitator, coordinate private support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Do not require private details. Record the actual preference, intervention, and confirmed or pending contact.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Language support does not meet the task

Scenario 12 · Adult PHP · Planned skill: DBT Describe

Notice

The arranged interpreter connection fails. Rosa reports not understanding. A willing peer and conversational English are not clinical interpretation.

First response

Through a working communication route: “The connection failed. I need to fix how the instructions reach you. You don’t need to explain personal information to the group.” If no qualified route is working, give only a brief, simple message that you are restoring the interpreter; no clinical instructions and no peer interpreting.

Choose the next step

• Qualified support reconnects: Confirm arrangements, address Rosa directly, use short segments, and check one instruction.

• Still unusable: Request backup language services. Do not count watching an unintelligible model as equivalent access.

• Peer volunteers: Thank them; use the program’s qualified route rather than assigning a patient to interpret.

• Rosa prefers peer help: Acknowledge the preference and consult the appropriate access process; do not improvise an exception.

• Possible harm or sudden clinical change: Use local assessment or emergency procedures immediately as indicated, requesting language support alongside the response. Do not wait for the worksheet or connection.

Return to Describe

Confirm usable communication or an appropriate alternative. Use a separate envelope example: describe its exterior without guessing its contents. Offer supported practice or an accessible model without requiring private material.

Support and record

With a co-facilitator, coordinate access support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Record actual language support, responses, unresolved limits, and handoff responsibility.

Sources: Linehan (2015), mindfulness teaching notes; AHRQ, Tool 9. AHRQ language access

Standalone quick reference

Check-in has used the practice time

Scenario 13 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Only twelve minutes remain. Own the timing and revise the plan without blaming the last speaker or racing through every slide.

First response

“I let check-in run long. We have twelve minutes left. I’ll focus on one useful practice and leave time to close.”

Choose the next step

• Smaller objective fits: Name one step, offer brief practice or observation, and retain closing time.

• Member objects: Acknowledge lost practice time and identify a change within your authority. Do not defend the whole session.

• Additional topic can wait: Confirm an available route and what remains pending; do not invent follow-up.

• Possible urgent need (for example, “I don’t know whether I can get through tonight”): Thank them,

request staff now, and ask them to stay until staff connect. Ask only essential immediate safety questions within your role; arrange fuller assessment privately with coverage. Privacy must not delay an indicated urgent response. The end of check-in or the session is not a reason to delay. Do not finish practice first.

Return to One Mindfully

Address immediate needs and explain what will be omitted. Use a separate library-announcement example: notice a shift to meal planning and return to the current message. Offer an accessible format. Writing and reporting personal material are optional. Do not treat observing as confirmed practice.

Support and record

With a co-facilitator, coordinate individual support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Confirm handoff responsibility. Record actual content, reported or observed responses, and pending arrangements rather than the original agenda.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A comment targets identity

Scenario 14 · Adult IOP · Planned skill: DBT Validation

Notice

Terry invalidates same-sex families after Keisha mentions a partner. Stop the conduct without treating Keisha's objection as equivalent harm.

First response

"Keisha, you don't need to defend your family. Terry, that comment dismissed same-sex families, and I'm stopping it."

Choose the next step

• Statement withdrawn: Offer Keisha listening or feasible support. No response, thanks, or forgiveness is required.

• Belief debate begins: Restate the conduct boundary once; do not debate whose identity or family is legitimate.

• Comment repeats: Pause teaching and request staff support through local procedures. Do not improvise expulsion.

• Peers defend intent: Return to the conduct and support needs without compulsory identity disclosures.

• Threat, possible harm, or clinical change: Use local assessment or emergency procedures. Do not finish the skill exercise first.

Return to Validation

Stop the conduct, acknowledge impact, and address requested support before teaching resumes. Use a separate overlooked-volunteer example. Reflect the stated experience and check accuracy; Keisha's experience is not a required practice task for Terry.

Support and record

With a co-facilitator, coordinate individual support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Confirm follow-up responsibility. Record individual conduct and actual responses without identifying peers. Taking a comment back does not, by itself, establish that the harm has been repaired.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Laughter interrupts a contribution

Scenario 15 · Adult PHP · Planned skill: DBT Check the Facts

Notice

Jamal stops speaking after peers laugh. The interruption is observable; its intended target is uncertain. Take the impact seriously.

First response

“Jamal, the laughter interrupted you. You don’t have to continue your example.”

“Please stop the side conversation. We need one speaker.”

Choose the next step

• Aside stops: Offer finishing, passing, listening, or feasible support. No acceptance of an explanation or apology is required.

• Intent is disputed: Hear a brief account without debating Jamal’s feelings or demanding private messages as evidence.

• Disruption continues: Pause teaching and use local conduct and support procedures.

• Member wants to leave: Say you’re requesting support; do not block or follow. Stay with the group and use the Scenario 29 steps.

• Threat, urgent harm, or clinical change: Pause for local assessment or emergency procedures. Do not complete the worksheet first.

Return to Check the Facts

Resume only after the interruption stops and support requests have a workable response. Use a separate rain-forecast example to distinguish information from prediction, name the fictional person’s anxiety, and consider whether the emotion and its intensity fit the available facts. Do not use the skill to argue away actual harm.

Support and record

With a co-facilitator, coordinate support and room coverage. Alone, remain with the group and request assistance; never leave it unattended. Record the individual’s statements, choices, and actual response excluding unnecessary peer identifiers. Confirm who owns the follow-up; a quiet room hasn’t settled anything.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

Members question a boundary's fairness

Scenario 16 · Adult IOP · Planned skill: DBT DEAR MAN

Notice

Devon questions check-in timing; peers join in. A legitimate concern and overlapping speech can occur together.

First response

"You're asking whether I used the same time limit for everyone. I'll hear one concern at a time."

Choose the next step

• Clear error: Own it and offer a feasible correction. Do not promise time you cannot provide.

• Unresolved comparison: Clarify current turns and the review route. Confirm who will follow up and when.

• Overlapping speech: Restore one speaker; pause teaching and request support if disruptive conduct continues.

• Private comparisons requested: Explain the general process without discussing another member's circumstances.

• Threat, urgent harm, or clinical change: Stop for local assessment or emergency procedures. Do not finish the exercise first.

Return to DEAR MAN

Resume after clarifying the limit and responding to the concern or arranging review. Use a separate choir member's request for written rehearsal details to practice selected DEAR MAN steps: Describe, Assert, Reinforce. A polished request is not required before a complaint can be heard.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, stay with the group and request assistance; never leave it unattended. Do not debate each angry statement or demand private explanations. Record the individual's concern, actual correction, and pending review excluding unnecessary peer identifiers. Silence isn't agreement.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Peers ask for graphic details

Scenario 17 · Adult IOP · Planned skill: DBT STOP

Notice

Maureen mentions a painful event; peers ask for details. Interrupt the pressure without implying that the experience cannot be acknowledged.

First response

“Maureen, you don’t need to describe what happened for us to take your experience seriously.”

“Please stop asking for details. Maureen doesn’t owe the group that account.”

Choose the next step

• Wants to listen: Honor the choice and agree on a usable support signal.

• Wants a full account now: Explain the format boundary and arrange an appropriate individual support route. Do not promise a treatment outcome.

• Peers persist: Restate the limit once. No debate, compulsory reassurance, or shared-trauma round.

• Concerning clinical change or urgent harm: Pause for local assessment or emergency procedures. Do not assume grounding is sufficient or finish the worksheet first.

Return to STOP

Resume only after detail-seeking stops and current support needs have a workable response. Use a separate ticket-machine frustration. A step back can be a pause in place; it does not require leaving. Do not make the disclosed event the practice task.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Confirm receipt and responsibility before describing a handoff as complete. Record actual choices and actions without unnecessary trauma details or peer identities.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Diagnosis or medication advice is requested

Scenario 18 · Adult PHP · Planned skill: DBT DEAR MAN

Notice

Walter reports feeling “wired” after a medication change and asks about diagnosis and the next dose. Treat the report as a clinical concern, not merely an off-topic question.

First response

“That change matters. I can’t decide a diagnosis or dose here. I’m contacting the appropriate clinical staff.”

Choose the next step

• Help available: Coordinate an individual review and confirm who accepts responsibility. Protect private details.

• Usual contact unavailable: Use the backup clinical route and local escalation plan; do not simply defer to a routine appointment. Before Walter leaves, confirm who will answer the dose question tonight and how to reach them.

• Peers give instructions: Stop advice about changing or sharing medication. Do not debate treatments.

• Significant clinical change or possible urgent harm: Pause for local assessment or emergency procedures. Do not finish a worksheet or test the skill first.

Return to DEAR MAN

Resume only when the current care response and coverage permit. Use a separate request for a written repair estimate. A well-worded request is not a prerequisite for clinical attention. This card gives no individualized instruction to take, skip, or change a dose.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Record the report, actual contact, and confirmed or pending handoff excluding unrelated peer histories. Do not infer cause or diagnosis.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

A guarantee is requested

Scenario 19 · Adult IOP · Planned skill: DBT Pros and Cons

Notice

Hannah asks for certain relief; a peer attributes success to effort. Answer honestly and stop the pressure.

First response

“You’re looking for something that will help tonight. I can’t promise how you’ll feel after using this.”

“We won’t judge another person’s effort or guarantee their result.”

Choose the next step

• Chooses to observe: Honor the choice without requiring belief or an endorsement.

• Earlier claim was inflated: Correct it directly; do not replace it with another guarantee.

• Demands certainty again: Restate the limit briefly and offer the task or support route. Do not debate each objection.

• Practical need identified: Arrange the appropriate support route; worksheet completion is not a condition for help.

• Urgent harm or clinical change: Pause for local assessment or emergency procedures. Do not finish the exercise first.

Return to Pros and Cons

Resume after clarifying the learning aim and addressing current support needs. Use a separate parcel-complaint example to consider immediate and later consequences. No private history is required, and instant relief is not promised.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended. Record actual choices, the member’s report of benefit or no benefit, and confirmed or pending follow-up excluding unnecessary peer identifiers.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

“We’ve done this already”

Scenario 20 · Adult IOP · Planned skill: DBT Effectively

Notice

Returning members question repetition; a newcomer withdraws. Check the concern without ranking members by experience.

First response

“You’ve seen this before. Let’s make the practice useful, while keeping the explanation available to everyone.”

Choose the next step

- Wants more challenge: Offer a manageable fictional application with competing goals, not extra busywork.
- Questions the schedule: Explain what you know, correct an actual error, and confirm any promised review.
- Starts instructing peers: Restore one speaker. No compulsory teaching, comparisons, or debate about expertise.
- Needs the basics: Keep the explanation available without requiring attendance or private-history disclosures.
- Urgent harm or clinical change: Pause for local assessment or emergency procedures. Do not finish the task first.

Return to Effectively

Resume with one shared aim and two supervised entry points. Use the borrowed-chair examples to compare actions with a chosen goal. Effectiveness is not obedience or a guaranteed outcome. Observation remains available.

Support and record

With a co-facilitator, coordinate task support and coverage. Alone, keep both options within the group you can supervise; never leave members or a subgroup unattended. Request assistance for individual support. Record actual task responses and confirmed or pending review excluding unnecessary peer identifiers. Familiarity is not mastery.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

Arrivals and departures interrupt teaching

Scenario 21 · Adult PHP · Planned skill: DBT STOP

Notice

Dana arrives during a demonstration while another member is called out. Multiple recaps begin. Address coordination and access without assigning blame.

First response

“Welcome, Dana. You don’t need to explain the arrival here.”

“Please hold the recaps. I’ll give the current instruction; we won’t repeat personal examples.”

Choose the next step

• Brief entry: Offer observation or the next fictional practice and an accessible task prompt.

• Scheduled departure: Confirm the receiving staff member and actual transition arrangement through the established process.

• Repeated traffic: Pause teaching and coordinate support; do not restart private check-ins.

• Criticism begins: Hear the scheduling concern without demanding private reasons or debating accommodations.

• Urgent harm or clinical change: Stop for local assessment or emergency procedures. Do not finish the worksheet first.

Return to STOP

Resume after responsibility, current-task access, and coverage are clear. Use a separate replacement-bulb frustration. A step back can be a pause in place; it does not require leaving. Nobody’s arrival or appointment becomes practice material.

Support and record

With a co-facilitator, coordinate transitions and coverage. Alone, remain with the group and request assistance; never leave it unattended or send a peer as staff escort. Record actual entry and participation excluding unrelated peer histories. Do not infer full attendance or completed assessment.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Membership changes unexpectedly

Scenario 22 · Adult PHP · Planned skill: DBT Validation

Notice

Rory expected a familiar practice partner. The person is absent; peers speculate. This card addresses membership uncertainty, not a live walkout.

First response

“You expected to practice together. I can’t share another person’s care or plans, but I can explain today’s options and support route.”

Choose the next step

• Needs a task option: Offer supervised observation, a fictional demonstration, or a feasible pairing. No immediate replacement relationship is required.

• Questions continue: Restate the privacy boundary briefly; do not debate or imply a private reason.

• Outreach proposed: Do not organize contact, share details, or recruit peers to investigate. Use the program’s actual boundaries.

• Wants support: Request appropriate staff contact and confirm responsibility.

• Urgent harm or clinical change: Pause for local assessment or emergency procedures. Do not finish an exercise first.

Return to Validation

Resume after stopping speculation, clarifying task arrangements, and addressing current support needs. Use a separate changed-walking-routine example. Nobody must disclose a personal loss, speculate about motives, or announce closure.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, stay with the group and request assistance; never leave it unattended. Record the present individual’s statements and choices without identifying the absent member or speculating about care. A request is not a completed handoff.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Practice increases distress

Scenario 23 · Adult IOP · Planned skill: DBT Observe

Notice

Mei says body-focused attention makes distress worse and asks to stop. Honor the request before explaining the method.

First response

“Yes, we’re stopping. You don’t need to keep focusing on your chest.”

“Please don’t coach Mei to continue.”

Choose the next step

• Wants to listen: Allow listening without another exercise or a private-history explanation.

• Wants an alternative: Offer a brief neutral external observation only if acceptable. Stop if unwanted or unhelpful; no endurance target.

• Chest pain, trouble breathing, or feeling faint: Treat as a possible medical emergency and use local emergency procedures now.

• Wants support: Use the established staff route and confirm who receives responsibility.

• Significant clinical change or urgent harm: Stop for local assessment or emergency procedures. Do not assume panic or dissociation, finish the worksheet, or try grounding first as a requirement.

Return to Observe

Resume only after ending the original practice and responding to current needs. Demonstrate noticing a clothespin without compulsory touch or internal focus. Do not promise calming. Observation of the demonstration or declining it remains available.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended or assign peers to monitor Mei. Record the report, stop request, choices, and actual or pending contact without inferred diagnoses or peer histories. Quietness is not assessment or proof of improvement.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

The room is difficult to tolerate

Scenario 24 · Adult PHP · Planned skill: DBT Observe

Notice

Screen flicker and corridor noise interfere with participation. Address the environment before turning discomfort into a practice task.

First response

“I’m pausing the instructions while we address what we can. You don’t need to explain a diagnosis.”

Choose the next step

• Controllable barrier: Reduce it and provide an accessible current prompt. Check what remains difficult.

• Space still unsuitable: Keep teaching paused and coordinate staff support. Confirm destination, access, and supervision before moving.

• Different access needs: Explore concrete options without a group vote on legitimacy or compulsory medical disclosure.

• Peers prescribe endurance: Stop the pressure; do not debate normal tolerance.

• Clinical change, urgent harm, or environmental emergency: Use local assessment or emergency procedures. Do not finish the worksheet first. Closing a door is fine; never lock or obstruct an exit.

Return to Observe

Resume only when the task is usable and current support needs are addressed. Offer a separate paper-windmill demonstration with a spoken description. No compulsory touch, closed eyes, or attention to distressing noise; no promise of relief.

Support and record

With a co-facilitator, coordinate environmental support and coverage. Alone, remain with the group and request assistance; never leave it unattended to find another room. Record the member’s report, actual changes, remaining barriers, and pending arrangements excluding unrelated peer histories or inferred diagnoses.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A suggestion does not fit real constraints

Scenario 25 · Adult PHP · Planned skill: DBT Problem Solving

Notice

The suggested home practice assumes private time and space that Carlos lacks. Acknowledge the mismatch before offering more advice.

First response

“I suggested quiet time you don’t have. That plan doesn’t fit. Would trying a step here help, or would you prefer our made-up example?”

“We can use a fictional example; you don’t need to explain private circumstances.”

Choose the next step

• Wants adaptation: Offer a feasible in-session step; do not assume a shorter home task solves the barrier.

• Declines public brainstorming: Stop advice and honor the choice without labeling resistance.

• Requests resources: Use the actual support route and verify availability before promising anything.

• Peers insist: Restate the permission and privacy boundary. No debate, financial proof, or group vote.

• Current harm or clinical change: Pause for local assessment or emergency procedures. Do not finish the worksheet first or use coping to bypass an unsafe situation.

Return to Problem Solving

Resume after correcting the assumption and addressing current support needs. Use a separate seed-exchange labeling problem with real constraints. No personal housing, family, or financial disclosure is required.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended to find resources. Record the relevant barrier, actual choice, and confirmed or pending contact excluding unrelated peer histories. A written plan does not prove feasibility.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

A legitimate concern about the intervention

Scenario 26 · Adult IOP · Planned skill: DBT Check the Facts

Notice

Remy correctly identifies that the facilitator assumed a distortion before hearing the situation. Answer the criticism before defending the method.

First response

“You’re right. I called it a distortion before I’d heard what happened. I shouldn’t have started there. We can leave your example out.”

Choose the next step

• Wants fiction: Use a genuinely separate example, not a disguised personal account. Observation or passing remains available.

• Requests review: Explain the actual route and confirm responsibility; skill completion is not a condition.

• Remains angry: Hear the concern and state the next process briefly. Do not debate every accusation or demand reassurance.

• Current harm or clinical change: Take the report seriously; pause for support, local assessment, or emergency procedures. Do not argue harm away or finish the worksheet first.

Return to Check the Facts

Resume after correcting the framing and responding to review or clinical needs. Use a separate ferry-timetable example. Identify the fictional traveler’s worry, check the current departure information, and consider whether the emotion and its intensity fit the facts. Checking may confirm that a concern is accurate; it does not presume the member is wrong.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, stay with the group and request assistance; never leave it unattended. Record the relevant facilitator action, individual response, and actual or pending review excluding unnecessary peer identifiers. An apology opens the repair; it doesn’t finish it.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

The facilitator applies a boundary inconsistently

Scenario 27 · Adult IOP · Planned skill: DBT DEAR MAN

Notice

The facilitator checks a personal notification after asking members to put phones away. Logan's objection identifies a real inconsistency.

First response

"You're right. I checked a personal notification while you were talking. I'm putting the phone away. Would you like to finish?"

Choose the next step

• Wants to continue: Return the turn without demanding acceptance, gratitude, or reassurance.

• Reports a pattern: Hear a brief concern and explain the actual review route. Do not debate each earlier event.

• Peers defend staff: Stop the pressure; members do not need to defend or forgive you.

• Private exceptions questioned: Explain the general request process without identifying another person's arrangements.

• Urgent harm or clinical change: Pause for local assessment or emergency procedures. Do not finish the worksheet first.

Return to DEAR MAN

Resume after correcting the behavior and addressing current review or clinical needs. Use a separate history-club request for advance slides. A complaint need not follow a skill formula before it is heard, and Logan need not rehearse it publicly.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, stay with the group and request assistance; never leave it unattended. Record the relevant facilitator action, individual response, and confirmed or pending review excluding unrelated peer histories. A member who keeps participating may not trust you again yet.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Standalone quick reference

Staff give conflicting directions

Scenario 28 · Adult PHP · Planned skill: DBT Effectively

Notice

One facilitator permits fiction or observation; another requires personal disclosure. Ren asks which instruction applies. Staff must clarify the task.

First response

“We gave you two different instructions. That’s ours to sort out. We’re pausing; nobody needs to share a personal example while we do.”

Choose the next step

• Agreement possible: State one coordinated instruction, participation choices, and the lead voice. Do not pretend the directions were identical.

• Disagreement continues: Keep the exercise paused and use the staff clarification route. No public status debate or member vote.

• Disclosure concern: Do not require private material to settle the conflict. Clarify this task without broad promises about program consequences.

• Clinical concern identified: Stop for local assessment or emergency procedures as indicated. Do not finish the worksheet or treat optional practice as a substitute for clinical response.

Return to Effectively

Resume only after roles and instructions are usable and current needs are addressed. Use a separate display-table teamwork example. Members do not portray staff or mediate the real disagreement.

Support and record

With two facilitators, confirm who covers the room before separate coordination; never both leave. Alone, remain with the group and request clarification or assistance; never leave it unattended or send peers to arbitrate. Record the individual’s question, actual clarification, and pending decisions excluding unrelated peer histories. Confusion is not treatment refusal.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member leaves during tension

Scenario 29 · Adult IOP · Planned skill: DBT STOP

Notice

Brianna leaves; destination and staff contact are unknown. No explicit threat was stated. Do not infer safety or dangerousness from departure alone.

First response

Pause teaching and activate the established departure/support route promptly.

“Please stay with me while I coordinate staff support. Peers don’t need to follow.”

Choose the next step

• Alone: Remain with the group; never leave it unattended to search. Communicate observations and last known location to staff.

• Co-facilitator present: Explicitly allocate room coverage and the departure response. Never both follow.

• No response: Use the backup or escalation route. Continue coordinating the response and group coverage until named staff explicitly accept the next responsibility and its timing.

• Contact confirmed: Clarify accepted responsibility and needed next actions. Making contact is not an assessment, and it doesn’t clear anyone to return.

• Known urgent risk, possible immediate harm, or clinical change: Activate local assessment or

emergency procedures without waiting for a return or finishing the worksheet.

Return to STOP

Resume only when response responsibility and current clinical needs permit, with coverage maintained. Otherwise keep teaching paused and escalate. Use a separate cabinet-painting frustration. A step back can occur in place; it does not require leaving.

Support and record

No peer search, private-history explanation, or public motive debate. Do not improvise physical blocking. Record observed statements, actual contacts, and unresolved status without declaring safety or disposition that was not assessed.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Returning after individual support

Scenario 30 · Adult PHP · Planned skill: DBT Describe

Notice

Micah returns with staff; peers ask for details and an apology. Coming back to the room doesn't mean a care plan is in place.

First response

"You don't need to explain the individual conversation. We're pausing while staff confirm the return arrangement."

Choose the next step

• Plan confirmed: Clarify accepted responsibility, necessary support, and feasible participation choices through the appropriate channel.

• Plan unclear: Keep teaching paused and seek clarification. A member's "I'm fine" is not the handoff.

• Peers demand repair: Stop intrusive questions and compulsory apology or forgiveness. Offer appropriate concern and support routes without debating private care.

• Conduct resumes: Apply the current boundary and local response.

• New clinical change or urgent harm: Stop for local assessment or emergency procedures. Do not finish a worksheet or require grounding first.

Return to Describe

Resume only after coordination and current needs permit. Use a separate market-stall layout, with accessible description. Do not make Micah's behavior the example or use practice as a readiness test.

Support and record

With a co-facilitator, allocate coverage and coordination explicitly. Alone, stay with the group and request assistance; never leave it unattended for a handoff conversation. Record the actual plan, role acceptance, and observed choice excluding unrelated peer histories. Quiet or an apology doesn't mean things are safe or repaired.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A disruption recurs across sessions

Scenario 31 · Adult IOP · Planned skill: DBT Problem Solving

Notice

Kevin repeatedly gives advice during others' turns. Address this instance and arrange review of the pattern; do not diagnose intent.

First response

"Kevin, please hold the idea while the speaker finishes. The speaker keeps the turn."

Choose the next step

• Stops: Restore the turn. Offer an accessible way to hold an idea without promising time for every suggestion.

• Pattern persists: Arrange private review of observations, instructions, barriers, and the member's account. Agree on a feasible cue, alternative, and review point.

• Current disruption continues: Pause teaching and request staff support; no public argument or improvised punishment.

• Peers complain: Hear the effect on participation without recruiting monitors or disclosing an individual plan.

• Urgent harm or clinical change: Stop for local assessment or emergency procedures. Do not finish the worksheet or require cue-following first.

Return to Problem Solving

Resume after protecting the current turn and addressing immediate needs. Use a separate repair-café tool-tracking problem. Members do not analyze Kevin or design consequences. A trial and review point do not guarantee resolution.

Support and record

With a co-facilitator, coordinate support and coverage. Alone, remain with the group and request assistance; never leave it unattended for individual review. No compulsory private-history explanation. Record actual behavior, the agreed cue if one exists, response, and pending review excluding unnecessary peer identifiers or character labels.

Skill source: Linehan (2015), emotion-regulation teaching notes. Guilford teaching notes

Standalone quick reference

Ending while tension remains

Scenario 32 · Adult PHP · Planned skill: DBT STOP

Notice

Four teaching minutes remain. Emery says the concern is unresolved. The lesson can end while necessary care and coverage continue.

First response

“We’re near the end, and this still needs attention. I’m checking who can hear your concern next.”

Choose the next step

• Nonurgent follow-up accepted: Confirm the receiving person and next step. Do not promise agreement or resolution.

• Contact unconfirmed: Maintain coverage, continue coordination, and use the backup/escalation route. Do not abandon responsibility at the scheduled endpoint.

• Debate restarts: State the limit briefly; no compulsory apology, forgiveness, private disclosure, or positive takeaway.

• Appointment or transport transition: Coordinate actual arrangements through local procedures; this card does not authorize detention or release.

• Urgent harm or clinical change: Stop routine closure for local assessment or emergency procedures. Do not finish a worksheet, require a skill, or wait for unaccepted handoff first.

Closing STOP reminder

Offer only if current care and transition responsibilities permit; otherwise omit it. Use a separate mislabeled-photo-album example. A step back can occur in place. The reminder is not a readiness or discharge test.

Support and record

With a co-facilitator, allocate coverage and coordination. Alone, remain with the group and request assistance; never leave it unattended. Record actual concerns, accepted or pending responsibility, and subsequent actions excluding unrelated peer histories or unsupported safety conclusions.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A member says they plan to end their life

Scenario 33 · Adult IOP · Planned skill: DBT Self-Soothe

Notice

Daniel describes an intention, a time frame, and a method. Stop the lesson. An apology or “I was joking” does not lower the urgency.

First response

“Daniel, thank you for telling me. I’m taking this seriously, and I’m getting a clinician here now. I’d like you to stay with me until they arrive.”

Choose the next step

• Urgent risk statement: Request a clinician now with Daniel’s exact words. Stay with Daniel and the group. If nobody answers, use the backup route.

• Takes it back: Keep the urgent response active; do not debate or defer contact until later in the day.

• Stands to leave: Ask him to stay; do not block or follow. Tell staff the direction, words, and description. Follow your emergency procedure.

• Says he already took something: Medical emergency. Use your emergency procedure now.

• Peers react: Say support has been requested, unless a handoff is confirmed. Attend to other requests without public disclosure.

Return to Self-Soothe

Only after a clinician has accepted responsibility and the group is covered. Ending or shortening the session is acceptable. If you continue, use a brief, optional fictional example and close with how to reach staff today.

Support and record

Never leave the group unattended or send a peer for help. Ask only essential immediate safety questions; no public detailed assessment or promise of secrecy. Record exact words, timing, actual actions, and accepted or pending responsibility. A delivered message or acknowledgement is not a handoff. Confirm the receiving person, the assessment or response responsibility accepted, the next action, timing, and ongoing group coverage.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A member appears intoxicated

Scenario 34 · Adult PHP · Planned skill: DBT Observe

Notice

Shannon has slurred speech, unsteadiness, and alcohol odor. Other medical causes are possible. New or sudden speech/balance changes or reduced responsiveness require emergency response now.

First response

“Shannon, I am concerned about your speech and balance. Please stay here while I get medical help now.” New or sudden signs, reduced responsiveness, or abnormal breathing use the emergency route immediately.

Choose the next step

• Accepts help: Report signs and known onset; request the indicated response now. Confirm receipt and responsibility; use the backup route if unanswered.

• Loud or argumentative: Do not debate. Keep your distance. “I’m getting someone to check on you.” Do not block the door.

• Reduced responsiveness or abnormal breathing: Activate emergency medical response immediately. Maintain supervision; provide only assistance for which you are trained.

• Mentions driving: Tell responding staff immediately; transport decisions follow program procedure.

• Peers comment: “I have requested staff support.” Report accepted responsibility only when confirmed; continue supervising the group.

Return to Observe

Only after a clinician has accepted responsibility. Use a neutral object, such as a glass of water. Do not refer to Shannon or the interruption.

Support and record

Maintain group coverage; never send peers for help. Record observations, exact words, times, accepted or pending responsibility, and transport concerns. Do not infer intoxication or a diagnosis from appearance alone.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member threatens another member

Scenario 35 · Adult IOP · Planned skill: DBT STOP

Notice

Cameron threatens Victor and steps toward him. This is a safety situation. Stop the lesson.

First response

“Cameron, please give Victor space. I’m pausing the group and getting help now.”

Choose the next step

• Sits down: Thank him briefly. Request help anyway. Keep Cameron and Victor apart.

• Continues or moves closer: Use your emergency procedure or panic alert now. Move others away calmly. Do not step between people or touch anyone.

• Physical aggression or weapon: Emergency. Follow your procedure, including 911 if that is the local instruction. Help others leave if safe.

• Victor wants to leave: Support it and tell staff.

• Peers react: Give the safe-spacing or evacuation direction that fits the danger; do not insist on remaining seated.

Return to STOP

Only after staff accept the response, coverage is arranged, and the setting is suitable. A supervised pause or ending teaching is acceptable. If appropriate, use a separate irritated-email example.

Support and record

Keep exits clear. Do not improvise restraint or blocking. Record the threat, timing, actual response, and accepted or pending responsibility. Notify the clinical lead immediately; applicable protective duties vary by jurisdiction.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

A virtual member hints at danger, then disconnects

Scenario 36 · Adult IOP · Planned skill: DBT STOP

Notice

Lisa says, “I’m not sure I’ll still be here tomorrow,” then leaves the meeting. Treat it as urgent until you know otherwise.

First response

“We’re pausing here. I’m staying with you while I arrange help. If you need support too, let me know.” Activate help promptly; do not wait to finish a callback.

Choose the next step

• Reached: Arrange clinical contact now through a private channel, confirm location, and allocate group coverage. A detailed call must not be audible to peers.

• No answer: Alert staff now with Lisa’s words, the time, and the confirmed location. Follow the emergency procedure.

• Rejoins: Arrange clinical contact now; do not postpone until after group. Confirm who accepts responsibility and covers the group.

• Members want to help: “Please do not message Lisa on the group’s behalf. I am coordinating the staff response.”

Return to STOP

Only when immediate needs, accepted responsibility, and group coverage permit. Use a separate fictional printer-jam example, or continue a supervised pause.

Support and record

Maintain active supervision, not just a camera image. Confirm location and backup number each session. No peer outreach. If assistance is unanswered, use the backup/emergency route. Record statements, times, and accepted or pending responsibility.

Skill source: Linehan (2015), distress-tolerance teaching notes. Guilford teaching notes

Standalone quick reference

Someone else is in a virtual member's room

Scenario 37 · Adult IOP · Planned skill: DBT Describe

Notice

Another person is heard in Omar's room. Other members' privacy and Omar's possible safety are both at stake.

First response

"Let's pause sharing while we check the connection and privacy arrangements." Avoid publicly identifying the other person or demanding an explanation.

Choose the next step

• Privacy restored: Confirm that others cannot hear group audio, see the screen, or overhear responses. Headphones alone may not resolve all three.

• No privacy: Pause personal sharing and arrange suitable access under program procedures. Muting does not stop incoming audio or screen exposure.

• Possible danger: Use an agreed low-disclosure signal or safe channel; do not assume private chat is unseen. Activate indicated support with the confirmed location.

• Other person intervenes: Use appropriate host controls and staff support. Do not confront them; coordinate safe contact before avoidable disconnection.

Return to Describe

Resume personal sharing only with suitable privacy and accepted safety-response responsibility. A brief fictional kitchen-counter task can support a supervised pause; it does not cure ongoing exposure.

Support and record

Maintain group supervision. No public private-history demands. Never send peers to investigate. Escalate urgent danger without waiting for the exercise; use backup help if unanswered. Record observations, privacy changes, actual contacts, and what remains unresolved.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A member joins while driving

Scenario 38 · Adult IOP · Planned skill: DBT One Mindfully

Notice

Patricia joins from a moving car. Safety and privacy come first.

First response

“Patricia, stop participating while driving. Rejoin only when safely parked in a suitable private place. Don’t type or adjust the device while driving.”

Choose the next step

• Rejoins: Confirm safe parking and suitable privacy before participation. Apply actual attendance rules; do not promise credit.

• Stays on: End the group connection using the established host process. Do not prolong discussion or request a typed reply while driving.

• No feasible location: Refer the access barrier for a realistic later arrangement. Do not promise availability or continue participation while driving.

• Camera off in a different, unconfirmed-driving situation: use the agreed private check and access plan. Do not request a typed reply from someone known to be driving. Camera status alone proves neither driving nor danger. A concerning statement or loss of contact uses Scenario 36.

• Peers comment: Give a brief process explanation without declaring Patricia safe.

Return to One Mindfully

Resume when driving participation has ended and any separate safety concern is addressed. Use an optional fictional cooking-and-phone example; do not make Patricia the lesson.

Support and record

Maintain supervision. No peer monitoring, private-history demand, or public argument. Activate an indicated urgent response before teaching. Record the report, times, host action, confirmed reentry conditions, and pending follow-up; a frozen image or traffic that seems stopped doesn’t prove the member has safely pulled over.

Skill source: Linehan (2015), mindfulness teaching notes. Guilford teaching notes

Standalone quick reference

A private chat message asks you to keep a secret

Scenario 39 · Adult IOP · Planned skill: DBT Validation

Notice

Brooke writes that home does not feel safe and asks for secrecy. Respond promptly through a safe channel; calm appearance does not resolve the concern.

First response

“Thank you for telling me. Can you safely use this chat now?” Use the agreed signal or safe channel and only essential immediate questions. Do not delay an indicated response for a reply.

Choose the next step

• Immediate danger: Activate the urgent response with location and message. Confirm responsibility, coverage, and safe contact; escalate if unanswered.

• Denies immediate danger: Arrange prompt clinical follow-up and safe contact. A yes-or-no reply is not a completed assessment; state only confirmed arrangements.

• Secrecy requested: Explain actual confidentiality limits. Share relevant information only through the appropriate clinical route; no promise of complete secrecy.

• Leaves or stops replying: Activate the Scenario 36 route and communicate monitoring concerns. Avoid repeated exposed messages or an unsafe callback.

Return to Validation

Resume only after accepted response responsibility, current needs, safe contact, and group coverage permit. Notification alone is insufficient. Otherwise maintain a supervised pause. Use a separate fictional example if teaching is suitable.

Support and record

Never read private messages aloud, send peers to investigate, or abandon group supervision. Use backup help for unresolved urgent concerns. Record exact words, actual actions, times, accepted or pending responsibility, and safe-contact arrangements in the approved clinical record.

Skill source: Linehan (2015), interpersonal-effectiveness teaching notes. Guilford teaching notes

Scenario 39: related SK-IE-02; Volume 1 p. 91; Volume 2 p. 26. Related relationship topic only; no dedicated Validation worksheet verified in Volume 2.

Handoff and group reset

A facilitator reference to adapt to your program. Keep patient information in the approved clinical record, not on this reusable page.

Make the request concrete

“I am [name/role] with [group/location]. I observed [behavior] and heard [exact words]. I need [appropriate response] now. I am maintaining group coverage. Please confirm who accepts the individual response and who covers the group.”

Close the communication loop

Confirm the receiving person, responsibility accepted, next action, and timing. If no one responds, use the backup or emergency route appropriate to the concern. A sent message, a return to group, or a quiet room does not complete assessment or transfer responsibility.

If the room can safely pause

State one brief direction without sharing private care information. Offer quiet observation or a simple optional task only if it does not divert needed attention. If safety, staffing, or privacy does not permit teaching, maintain the appropriate supervised pause or transition under program procedures.

Before resuming or ending teaching

Check current needs, accepted responsibility, coverage, and any follow-up requested by other members. Name unresolved arrangements honestly. Confirm who owns each next step; do not promise availability or closure.

Record and review

Record observations, exact relevant statements, interventions actually delivered, response, contact times, who accepted responsibility, and what remains pending. Use separate individual records and exclude unnecessary peer identifiers. Include safety-critical identifying details only when relevant, necessary, and permitted or required in the appropriate protected record. After an acute event, use the program’s reporting and supervision process; do not enter speculation as a finding.

Before the next group

Verify backup contacts, coverage, access arrangements, and safe communication. For virtual groups, verify current location, callback details, privacy, and disconnection response. Practice the staff contact route before relying on it.

Remote emergency routes

When coordinating a remote emergency, give the patient’s confirmed location and use the pre-established emergency route for that location. Keep patient addresses and telephone numbers in the approved clinical record, not on this reusable reference.

After an acute event is contained, assess what members and staff need and what coverage permits. Volume 1, SP-CD offers optional practical support; it is not a mandatory incident narrative or psychological debriefing. Do not require immediate disclosure or resume teaching simply because the schedule says to.